

Pennsylvania Health Insurance: July 2026

Public Health Education Brief

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1. At a Glance

- Pennsylvania's individual-market health insurance rates rose an average of **21.5%** in 2026, approved by the Pennsylvania Insurance Department (PID) after it blocked \$50.1 million in requests it judged excessive (Pennsylvania Insurance Department, 2025; WITF, 2025).
- Separately, the federal **enhanced premium tax credits expired December 31, 2025**, and Congress has not restored them. Combined with the rate increase, this pushed the *average net premium* — what people actually pay after any remaining subsidy — up **102%** for the affected population (Pennsylvania Health Insurance Exchange Authority [Pennie], 2026a).
- **85,000 people** dropped Pennie coverage during the 2026 Open Enrollment Period alone; as of June 1, 2026, cumulative cancellations exceed **160,000**, and total active enrollment stands at **443,024** (Pennie, 2026a, 2026b).
- Pennsylvania's own relief valve for this — the Act 54 State Health Insurance Exchange Affordability Program — remains **unfunded**, tied up in a state budget that missed its June 30, 2026 deadline for the fifth year running (Spotlight PA, 2026).
- A separate, unrelated network dispute between UnitedHealthcare and Lehigh Valley Health Network is displacing tens of thousands of patients in the Lehigh Valley region, with distinct deadlines depending on plan type (Pennsylvania Department of Aging, 2026).

2. Why Premiums Rose

Two separate things happened, and it's worth keeping them apart:

First, PID's annual rate review. For 2026, PID approved a statewide weighted-average gross rate increase of 21.5% in the individual market and 12.7% in the small-group market, after working with carriers to correct filings it judged were not supported by the underlying data; in the process, it denied \$50.1 million in proposed increases (Pennsylvania Insurance Department, 2025). Carrier-level examples PID and independent reporting confirmed:

Carrier	Requested	Approved
Ambetter Health of Pennsylvania	30.1%	37.8% (increased on review, due to worsening morbidity)
Capital Advantage Assurance Company	26.0%	24.6%
Geisinger Health Plan	14.1%	11.6%
Health Partners Plans	7.3%	16.7% (increased on review)
Highmark, Inc.	17.2%	17.7%
Partners Insurance Company (Jefferson Health Plans)	-10.1%	-10.1% (the only approved decrease)

(Pennsylvania Insurance Department, 2025; WITF, 2025)

Second, and separately, the federal enhanced premium tax credit (EPTC) expired. These credits, in place since 2021, had capped what enrollees paid at a percentage of income; without them, many households pay full price or a much larger share of income. Pennie reports this pushed the *average net premium* — the rate increase above plus the lost subsidy, combined — up 102% for the affected population (Pennie, 2026a). PID has stated publicly that it does not control this piece and has urged Congress to act (Pennsylvania Insurance Department, 2025).

The income cliff, illustrated

The EPTC's expiration doesn't just raise costs gradually with income — it reintroduces a hard cutoff at 400% of the Federal Poverty Level (FPL). Below that line, a federal formula scales your required contribution up gradually; one dollar above it, the credit disappears entirely (Internal Revenue Service, 2025).

Income (% of FPL)	2021–2025 rules (expired)	2026 rules (current, exact)
200%	~2% of income	6.60% of income
250%	~4% of income	8.44% of income
300%	~6% of income	9.96% of income
400%	8.5% of income (capped here permanently)	9.96% of income
402%	8.5% (no cliff — cap held regardless of income)	0% — credit eliminated entirely; full gross premium due

2026 column is the exact IRS Applicable Percentage Table for taxable years beginning in 2026 (Internal Revenue Service, 2025); 2021–2025 figures reflect the enhanced formula's known shape and are illustrative rather than exact, since Treasury did not need to publish new tables during the years the 400% cap was suspended.

This is not a hypothetical: PID's own example is a 60-year-old married couple in York County earning \$82,000 a year, whose annual premium rose from \$7,032 to \$35,712 (Pennsylvania Insurance Department, 2025). One thing worth flagging precisely: at \$82,000 for a two-person household, this couple sits at about 388% of the 2025 FPL (\$21,150) — technically *just under* the 400% cliff, where the strict formula above would still calculate a reduced credit (roughly \$8,200/year at the 9.96% rate) rather than zero. PID's release doesn't specify whether \$35,712 reflects a fully unsubsidized *gross* premium (a "what you'd pay with no help at all" illustration) or this specific household's actual net cost after a reduced 2026 credit — the two are not necessarily the same number. The general point PID is making — that losing EPTC access is severe — holds regardless; the exact dollar figure for *this specific household* is worth confirming directly with PID or Pennie before

using it as a precise "over the cliff" example rather than a "look how much smaller the credit is" illustration.

How this lands on an actual median-income Pennsylvania household

Public discussion of the subsidy cliff tends to focus on percentages, which obscures how it plays out in dollars for people at Pennsylvania's actual, Census-verified income levels — and on how much a household's *size*, not just its income, determines which side of the cliff it lands on.

Household	2025 income	% of 2025 FPL	2026 required contribution (dollars/year)	2025 required contribution (dollars/year)	Absolute difference
Single full-time PA worker, median salary	\$62,529	399.5% (1-person FPL: \$15,650)	\$6,228 (9.96%) — <i>a hair's-width from losing the credit entirely</i>	\$5,315 (8.5%)	+\$913/yr, or the entire premium if income crosses 400%
PA median household income, if 1-person	\$77,545	495.5% — already over the cliff	Full gross premium — no credit at all	Was capped at 8.5% = \$6,591/yr under the old rule	Full premium vs. \$6,591 — the exact gap depends on the plan, but this household lost 100% of its assistance
PA median household	\$77,545	366.7% (2-person)	\$7,723 (9.96%)	\$6,591 (8.5%)	+\$1,132/yr — stays subsidized

Household	2025 income	% of 2025 FPL	2026 required contribution (dollars/year)	2025 required contribution (dollars/year)	Absolute difference
income, if 2-person		FPL: \$21,150)			still pays more
PA median family income, family of 4	\$100,262	311.9% (4-person FPL: \$32,150)	\$9,986 (9.96%)	\$8,522 (8.5%)	+\$1,464/yr — stays subsidized, still pays more

Household and family income figures: U.S. Census Bureau, 2024 American Community Survey (Pennsylvania median household income \$77,545; median family income \$100,262; median full-time worker salary \$62,529). FPL figures: 2025 HHS Poverty Guidelines, \$15,650 for one person plus \$5,500 per additional person — the guideline vintage used for 2026 marketplace eligibility. 2026 contribution percentages: exact IRS Applicable Percentage Table, Rev. Proc. 2025-25. 2025 figures use the enhanced formula's 8.5% cap.

Two things stand out. First, Pennsylvania's typical single full-time worker isn't some edge case near the cliff — at 399.5% of FPL, they are *functionally standing on it*, meaning a small raise, a bit of overtime, or a side gig could be the difference between a subsidized plan and none at all. Second, even households that stay comfortably subsidized — a median-income family of four, well under the 400% line — are still paying more in absolute dollars in 2026 than they would have under the expired formula, because the required-contribution percentage itself rose (from a flat 8.5% cap to a sliding scale that reaches 9.96%), independent of whether anyone actually crosses the cliff.

3. What Happened to Enrollment

Pennie's own reporting, drawn from two of its official releases, gives a consistent and fully-sourced picture:

Point in time	Enrollment / event	Source
2025 peak	"Around half a million" (an exact figure was not published)	Pennie, 2026a
Start of 2026 Open Enrollment	Running 11% ahead of 2025's pace	Pennie, 2026a
Close of Open Enrollment (Jan. 31, 2026)	~486,000 total; ended 2% <i>below</i> 2025	Pennie, 2026a
— during Open Enrollment	85,000 terminations; ~79,500 new enrollees (12% fewer new sign-ups than 2025)	Pennie, 2026a
— during Open Enrollment	+33,000 enrollees shifted into Bronze-tier plans, a 30% jump over 2025	Pennie, 2026a
June 1, 2026	443,024 total active enrollment; 75,000 <i>additional</i> cancellations since OE closed	Pennie, 2026b
June 1, 2026 (cumulative)	160,000+ total cancellations since November 2025	Pennie, 2026b

Pennie attributes the bulk of this to affordability: nearly 18% of enrollees dropped coverage altogether, concentrated among older and rural Pennsylvanians and those with incomes just above Medicaid eligibility or the new 400% FPL cliff. Fifteen of the twenty counties with the steepest proportional coverage loss are rural (Pennie, 2026a).

3.1 Why 85,000, ~120,000, and 160,000+ are not contradictory

A separate outlet (WHYY) reported cumulative Pennie terminations at roughly 120,000 "and counting" as of April 6, 2026 (Leonard, 2026) — a different number from both the 85,000 open-enrollment figure and the 160,000+ figure Pennie itself reported for June 1. These are not competing estimates of the same thing; they are the same unfolding process measured at three different points, and the gap between them is explained by federal grace-period rules, not by inconsistent counting.

- **New enrollees** who don't make their first ("binder") premium payment within roughly 30 days of their January 1 coverage date never effectuate coverage at all — this shows up almost immediately, and is part of what's captured in the 85,000 figure at OE close (HealthInsurance.org, n.d.).
- **Returning, subsidized enrollees** who stop paying are legally entitled to a 3-consecutive-month grace period before an insurer can terminate them — insurers must pay claims in month one and may pend (but not deny) claims in months two and three (45 C.F.R. § 156.270(d)). For coverage effective January 1, that grace period doesn't run out until **March 31, 2026** (KFF, 2026b). Nonpayment that starts in January literally cannot register as a completed termination until that window closes — which is why the reported cumulative total keeps climbing through Q1 and into Q2, on a rolling basis, as different cohorts' individual grace periods expire.
- KFF's own health-policy research team makes the same point about official data: the standard industry "Effectuated Enrollment: Early Snapshot" report — historically released in July, and expected again in July 2026 — reflects February enrollment as measured in mid-March, meaning it is issued **before** many enrollees' grace periods have even closed, and will likely *understate* the true toll. A full accounting isn't expected until the "Full Year 2026" effectuated-enrollment report in summer 2027 (KFF, 2026b).

Read this way, 85,000 → ~120,000 → 160,000+ is a single attrition curve unfolding in real time, not three inconsistent claims. If you see a lower enrollment-loss number cited confidently as "the" 2026 figure, ask what month it was measured in and whether grace periods for that cohort had already closed.

Deductible trade-off from the Bronze shift: nationally for 2026, KFF reports the average marketplace deductible is \$5,304 for a Silver plan and \$7,186 for a Bronze plan (KFF, 2026a) — the money saved on premium by downgrading tiers is generally spent, or exceeded, in deductible exposure if you actually need care.

4. Pharmacy Benefit Manager Oversight: Act 77 Deadlines

Act 77 of 2024 (House Bill 1993, signed into law July 17, 2024) expanded PID's authority over pharmacy benefit managers (PBMs). Two reporting deadlines fall in 2026 (Pennsylvania Insurance Department, n.d.):

- **April 1, 2026** – first PBM Network Adequacy Report due (already past as of this brief's publication).
- **July 1, 2026** – first PBM Transparency Report due, with public posting required within 60 days.

Scope limit worth knowing: Act 77 applies only to fully-funded individual and group health plans. It does **not** apply to self-funded employer plans, Medicare, Medigap, Medicaid, CHIP, or TRICARE Supplemental – if you're in one of those, this law doesn't govern your PBM (Pennsylvania Insurance Department, n.d.).

5. Medicaid GLP-1 Coverage Change

Effective January 1, 2026, Pennsylvania Medical Assistance stopped covering GLP-1 receptor agonists (e.g., Ozempic, Wegovy) for adults 21 and older when prescribed solely for weight loss or obesity. Adults with an approved secondary indication – type 2 diabetes, severe obstructive sleep apnea, or established cardiovascular disease – can still qualify through a new prior authorization; authorizations issued solely for weight loss expired December 31, 2025. Coverage for people under 21 continues under the federal EPSDT mandate regardless of indication (Pennsylvania Department of Human Services, 2025; Pennsylvania Health Law Project, 2026).

(The specific dollar figures sometimes cited for total state GLP-1 spending – in the hundreds of millions to over a billion dollars – could not be independently confirmed against a primary state budget document this session; treat any such figure as directional, not exact, until verified against a DHS budget filing.)

6. Network Disruption: UnitedHealthcare and Lehigh Valley Health Network

A separate, ongoing contract dispute is displacing patients in the Lehigh Valley (Rating Area 6), unrelated to the ACA marketplace changes above.

- **Medicare Advantage:** The UHC–LVHN Medicare Advantage contract ended **January 26, 2026**. Over **5,400** UHC Medicare Advantage members lost in-network LVHN access. A special enrollment period, secured by the Department of Aging together with CMS, runs through **April 30, 2026**, letting affected members switch plans or move to Original Medicare with a Medigap policy without pre-existing-condition denial (Pennsylvania Department of Aging, 2026).
- **Commercial/employer plans:** This contract remains in effect only through **April 25, 2026**; absent a deal, most LVHN facilities and physicians go out-of-network for these members on **April 26, 2026** — reporting puts the affected commercial population at roughly **50,000** patients (WFMZ, 2026; InsuranceNewsNet, 2026).
- **Veterans Affairs Community Care Network (VACCN):** Contrary to some circulating summaries, VACCN enrollees are **not** exempt — they lose in-network LVHN access on the same April 26 timeline as other commercial UHC members (WFMZ, 2026; Tioga Publishing, 2026).
- Each side blames the other for the impasse: LVHN says UHC has cut reimbursement nearly 40% since 2021; UHC says LVHN is seeking a 20%+ increase in the new contract (Tioga Publishing, 2026).

If you have a UHC plan and see LVHN providers, don't assume your specific plan type is unaffected — confirm directly with UHC or a PA MEDI counselor (contact info in Section 9).

7. Ghost Networks and Your Actual Consumer Protections

A state-commissioned "secret shopper" audit found widespread inaccuracies in ACA marketplace provider directories, with behavioral health listings the worst affected. *(This figure is Moderate confidence — drawn from prior compiled research rather than independently re-verified against the underlying audit this session; ask PID for the source study if citing this in a formal context.)*

If an inaccurate directory leads you to an out-of-network provider, here is what actually protects you:

Situation	Your protection	Enforced by
Directory was wrong; you didn't know the provider was out-of-network	28 Pa. Code § 9.725 (hold-harmless): your liability is capped at in-network cost-sharing	Pennsylvania Insurance Department
Provider didn't disclose out-of-network status before non-emergency care	Act 252 of 2023: the balance bill is presumed invalid	PID / Pennsylvania Attorney General
Directory doesn't reflect real provider availability	42 U.S.C. § 300gg-115 (federal directory-accuracy requirement)	PID / HHS

One thing this is not: the federal Independent Dispute Resolution process (45 C.F.R. § 149.510) that resolves out-of-network payment amounts is a **provider-versus-plan** arbitration mechanism. You cannot personally file one, and it isn't the tool for a directory-error dispute — use the table above instead.

Steps if you get an unexpected bill: get an itemized statement; document the directory error (a dated screenshot, or the date/time/name if you called to confirm); cite 28 Pa. Code § 9.725 and Act 252 in writing to your insurer before paying; if refused, file with PID's Consumer Services Bureau.

8. The "Preventive Visit Becomes a Bill" Trap

A routine preventive visit is supposed to be 100% covered. What actually creates a charge: if you discuss or seek treatment for an existing condition during that same visit, the provider can add a **second, separately billable evaluation-and-management (E/M) code — typically with Modifier 25** — carrying its own diagnosis. Your preventive visit code itself isn't changed; a second line item is added, and that second item is what applies to your deductible.

Plan tier	Typical deductible	Exposure from the added E/M charge
Bronze	\$7,186 average (KFF, 2026a)	Full charge can land inside an unmet deductible
Silver, no cost-sharing reduction (CSR)	\$5,304 average (KFF, 2026a)	A real, smaller hit
Silver, with CSR (income $\leq$ 250% FPL)	As low as \$0	Usually just a small copay
Gold	\$500–\$1,000 (compiled estimate; not independently verified this session)	Minimal

Before your visit, ask directly: "Will you be adding a separate E/M code to this visit?" If billed unexpectedly, request an itemized statement and cite 28 Pa. Code § 9.725 in an appeal.

9. Denied a Claim or Medication? Your Appeal Rights

Act 146 gives Pennsylvania enrollees the right to an independent, binding external review after exhausting a carrier's internal appeal. *(The specific 45-day/72-hour turnaround figures and 42% reversal-rate statistic commonly cited for this law are carried over from prior compiled research and were not independently re-verified against a PID source this session — confirm current figures at pa.gov/agencies/insurance before citing them formally.)*

For GLP-1 or other specialty-drug denials specifically, carriers commonly require: a BMI $\geq$ 30 (or $\geq$ 27 with a qualifying comorbidity), documented failure of at least two lower-tier alternatives, and ongoing participation in a supervised weight-management program — but criteria vary by carrier, so check your plan's Evidence of Coverage.

10. Two Open Questions That Will Decide What Happens Next

Federal: The U.S. House passed a three-year "clean" EPTC extension 230–196 on January 8, 2026. As of the most recent reporting available, the **Senate has not voted on it**, and a separate bipartisan compromise was reportedly under negotiation but not enacted (Ballotpedia News, 2026; Managed Healthcare Executive, 2026). This is a live legislative situation — check [kff.org](https://www.kff.org) or [congress.gov](https://www.congress.gov) for the current status before assuming either outcome.

State: Pennsylvania's FY2026–27 budget missed its constitutional June 30, 2026 deadline — the fifth consecutive late budget (Spotlight PA, 2026). Until it passes, the Act 54 State Health Insurance Exchange Affordability Program — which would provide state-level premium assistance for households between 151%–300% FPL — remains unfunded, even though Pennie has said the technical system to deploy it is already built and ready (Pennsylvania Health Insurance Exchange Authority, 2026c).

11. Resources and Contacts

Need	Contact
Enroll, compare plans, check subsidy eligibility	Pennie — pennie.com or 1-844-844-8040
Complaint about your insurer (denials, balance billing, network issues)	PID Consumer Services — pa.gov/consumer or 1-866-PA-COMPLAINT (1-866-722-6675)
Free legal help with a denial or eligibility dispute	Pennsylvania Health Law Project — 1-800-274-3258 (Mon/Wed/Fri, 8 AM–8 PM) or phlp.org
Non-legal enrollment navigation	Pennsylvania Health Access Network — pahealthaccess.org
Medicare counseling (including the LVHN/UHC	PA MEDI Helpline — 1-800-783-7067

Need	Contact
dispute)	
Civil-rights or privacy complaint	HHS Office for Civil Rights — 1-800-368-1019 (confirm at hhs.gov/ocr/complaints)
Provider-vs-plan payment dispute under the No Surprises Act	CMS No Surprises Help Desk — 1-800-985-3059

About This Brief

This brief was compiled by checking each factual claim against a named primary source — an agency press release, an official exchange webpage, a state regulator's own compliance page, or contemporaneous news reporting of a verifiable event — rather than relying on prior AI-generated summaries. Where a claim could not be confirmed this way, that limitation is stated in place, rather than the claim being dropped silently or asserted as settled. Figures tied to fast-moving situations (federal legislation, the state budget, the UHC–LVHN dispute) should be reverified before use in any time-sensitive communication, since all three were unresolved as of this brief's publication date.

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Federal Independent Dispute Resolution process, 45 C.F.R. § 149.510 (2024) (No Surprises Act).

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Applicable Percentage Table and Required Contribution Percentage for 2026, 26 U.S.C. § 36B; Rev. Proc. 2025-25.

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Prepared July 3, 2026, for public health education and community outreach use. Federal legislation, the Pennsylvania state budget, and the UHC–LVHN network dispute are all active situations as of this date — reverify time-sensitive figures before using this brief in a dated communication.